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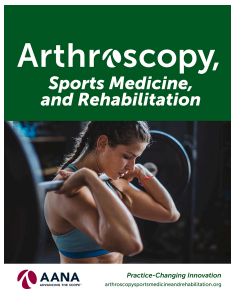


Medical Malpractice Lawsuits Against Team Physicians in the National Football League (NFL) Increased Between 1971 and 2020 But Remain Low Overall

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## ABSTRACT

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Purpose: To examine malpractice lawsuits by National Football League (NFL) players against team physicians to understand factors involved with medical liability in the context of professional football.

Methods: A comprehensive review of publicly available legal databases, court records, and news archives was conducted in August 2023 to identify and analyze malpractice lawsuits involving NFL players, teams, and team physicians. Data on case demographics, physician subspecialty, allegations, and outcomes, including monetary amounts, were collected and analyzed. Linear regression was utilized to assess for a temporal relationship with the number of lawsuits.

Results: There were 273 physicians identified, 190 of which were orthopaedic surgeons. There were 28 lawsuits from NFL players against physicians and a significant increase in lawsuits was identified from 1971-2020 ($p=0.0086$, $R^2= 0.60$). The most common anatomic locations of injury in a lawsuit were knee (n=11), spine (n=4), and foot/ankle (n=4). Other lawsuits include the result of wrongful death from heat-related injury as well as a class action lawsuit regarding pain medication prescribing practices and other individual lawsuits over anti-inflammatory use. The highest lawsuit amount filed was $180 million with at least six lawsuits filed for over $1 million, although many settlements and final verdicts were not available/sealed.

Conclusions: There was an increase in the number of medical malpractice lawsuits by professional athletes against team physicians in the NFL between 1971 and 2020, but the overall number remains low.

Clinical Relevance: Understanding the frequency and characteristics of medical malpractice lawsuits is essential for physicians, sports organizations, and policymakers to help guide appropriate healthcare delivery and risk management strategies for athletes in the NFL.

Key Terms: National Football League; malpractice lawsuit; orthopaedic surgeons; liability; legal

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## INTRODUCTION

The National Football League (NFL) is a popular professional football league in the United States (US), featuring 32 teams each with its own group of team physicians.[1,2]In a 2017 report, there were an average of 5.9 injuries per regular season game in the NFL, which is the highest

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among all US professional sports teams.[3] Furthermore, there has been an increase in the number of injuries per year from 5.3 injuries per game to 6.7 injuries per game from 2009 to 2015.[3] Team physicians serve several roles in addition to managing these on-field injuries, including preparticipation physicals, injury prevention protocols, rehabilitation oversight, return-to-play assessments and coordination with consulting physicians.[4] . As the success of major NFL franchises is intrinsically linked to the performance and health of key players, injuries are a major concern with far reaching consequences. The financial implications alone are noteworthy, with NFL teams collectively paying over half a billion dollars in the 2019 season to Week 1 starters who missed games as a result of injury and to players who ended the year on injured reserve.[5] The 2023 contract of Cincinnati Bengal's quarterback Joe Burrow, including an annual average salary of $55 million per year, highlights the substantial financial investment tied to players.[6] Consequently, financial and team successes place pressure on athletes and medical personnel to return players expeditiously, which is not always dependent on a complete recovery or return to a preinjury state.[7]

Financial incentives, such as loss of performance bonuses, may motivate players to return prematurely, in addition to players' competitive drive to contribute to the overarching success of the team. These factors may place the medical professional team responsible for the health and rehabilitation of injured players in challenging situations. Team physicians may find themselves navigating a delicate balance between team and individual short-term performance goals and the long-term well-being of the athlete, which may expose physicians to a range of legal liability as they deal with pressure to expedite recovery, while ensuring delivery of optimal medical care.

Recent high-profile medical malpractice lawsuits have brought these concerns to the forefront of team physicians and prospective team physicians minds.[8] However, there is limited

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data quantifying the prevalence of medical malpractice lawsuits against NFL team physicians. This gap in knowledge extends beyond professional football as well, with implications for medical practice across other elite amateur and professional sports leagues, which may have similar risks of substantial legal and financial liability.[9]

The purpose of this study was to examine malpractice lawsuits by National Football League (NFL) players against team physicians to understand factors involved with medical liability in the context of professional football. Our hypothesis is that there will be a significant increase in the number of medical lawsuits against team physicians in the NFL during the study period.

**METHODS**

A comprehensive review of publicly available legal databases, court records, and news archives was conducted in August 2023 by two independent reviewers (KL and JA) to identify and analyze malpractice lawsuits involving NFL players, teams, and team physicians. This included searches through publicly available search engines such as Google. An initial search was conducted using the name of each of the 32 NFL teams followed by the term "medical malpractice" or the term "lawsuit." Then, a search was conducted specifically on the respective orthopedic surgeons associated with each of the 32 NFL teams *(*Appendix I*)*. Terms used in this search included various combinations of the physician's name, the physician's associated team, and the term "malpractice" or "lawsuit." All mentions of medical malpractice were included in the data collected. Exclusion criteria included lawsuits against NFL teams or team physicians in which the plaintiffs were non-NFL players or non-medical related lawsuits. The primary outcome of interest was the total number of medical malpractice lawsuits filed by NFL players from 1971 to 2020. Secondary outcomes, when available, were also collected including case

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demographics, physician subspecialty, allegations, and outcomes, including monetary amounts. If a lawsuit was filed against more than one team physician or more than one team, it was recorded as an additional lawsuit. All data is presented in a descriptive format. Linear regression was utilized to assess for a temporal relationship with the total number of medical malpractice lawsuits, with a statistically significant p-value denoted as <0.05.

**RESULTS**

After a comprehensive search of all 32 NFL teams, there were 273 physicians total that were found to be associated with NFL teams, of which, 190 were orthopaedic surgeons with many fellowship trained in various subspecialties within orthopaedics. An in-depth review of publicly available information including legal databases, court records, and news archives of each of the orthopaedic physicians found 28 lawsuits from current or former NFL players between the years of 1971-202. There were two from 1971-75, zero from 1976-80, one from 1981-85, three from 1986-90, one from 1991-1995, four from 1996-2000, four from 2001-2005, four from 2006-10, three from 2011-15, and six from 2016-2 (Figure 1).

From the first 25 years (1971-1995) to the following 25 years (1996-2020), the rate of medical malpractice lawsuits increased from 0.28 (n=7) to 0.84 (n=21) lawsuits per year. Linear regression demonstrated a significant increase in lawsuits over this interval of time (p=0.0086 and $R^2$= 0.60).

Of the 28 lawsuits, the most common anatomic locations of injury were the knee (n=11), spine (n=4), and foot/ankle (n=4), followed by the shoulder (n=2), wrist (n=1), hip (n=1), lung (n=1), and other (n=4). Additional information on the reason for medical malpractice lawsuit, condition being and treated and outcome of the lawsuit is described in Table 1. Other lawsuits included the result of wrongful death from a heat related injury, in addition to to a class action

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lawsuit regarding pain medication prescribing practices and other individual lawsuits over anti-inflammatory use. The highest lawsuit amount filed was for $180 million with at least six lawsuits filed for over $1 million, although many settlements and final verdicts were not available/sealed.

**DISCUSSION**

From 1971 to 2020, there has been a statistically significant increase in the number of medical malpractice lawsuits filed against team physicians in the NFL. Furthermore, a total of 28 lawsuits have been identified over the last 5 decades with knee, spine and foot and ankle injuries being the most common reason for lawsuit. While the role of the team physician and sports medicine trained physician has expanded over the last several decades, injury patterns and treatment decisions are increasingly complex, with high expectations for successful recovery and the added stake of the level of exposure of professional athletes and scrutiny from the public eye. In this context, the results of this study highlight the increasing legal liability faced by team physicians responsible for the care of NFL athletes over the last five decades.

Since 2004, the NFL has banned teams from negotiating marketing contracts with specific medical groups.[2].[10] The intention was to ensure medical decisions were not swayed by conflicts of interests, however this has not necessarily assuaged players' apprehensions regarding the motives of team-employed physicians.[2-4] A 2013 survey, performed by the NFL Players Association, revealed that 78% of players lacked trust in team medical staff.[11] Alongside this, the growth of social media has intensified both scrutiny and distrust of physicians, particularly when caring for some of the most recognizable figures in the country, such as professional athletes.[12] Repairing this prevailing sense of medical mistrust is crucial not only for ensuring high-quality

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care, but also for fostering a patient-physician relationship built on trust, which in turn can mitigate the litigious nature of caring for these athletes.

The Tort law applies to sports medicine with the same negligence standards that apply to the practice of general medicine when used to evaluate medical care provided to athletes in a sports setting.[13, 14] Courts do not recognize sports medicine as a separate medical specialty with its own standard of care. As many different specialists serve as team physicians (orthopaedic surgeons, internists, family practitioners, physical medicine and rehab physicians, etc.), liability is determined on a case-by case basis with the standard of care measured against the standard of "good medical practice". Major concerns in terms of medical liability for team physicians include preparticipation examinations, determination of medical eligibility, evaluation of significant on-field injuries, information disclosure and privacy. [13, 14]

In caring for high-level athletes, it is important to understand that the patient-physician relationship is seldom isolated to the physician and athlete. More often, it includes discussions with family members, agents, coaches, managers, and other team personnel. Upfront and direct communication, initiated by the physician where appropriate, is fundamental to building trust and ensuring high-quality care. Although various parties may attempt to drive decision making[15], physicians must prioritize the athlete's best interests, maintain ongoing communication with all involved parties, and document these discussions thoroughly.

Emerging legal issues relevant to the team physician include concussion protocols and the use of prescription drugs. Although only 1 of the 28 lawsuits from our findings were the result of a head injury, greater emphasis has been placed on properly managing concussion return to play protocols. With a rate of 0.62 concussions per regular season game in the NFL, the highest among all US professional teams, this issue is not only relevant for the neurologists and

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neurosurgeons, but all sideline team physicians.[3] In addition, improperly prescribing and/or overprescribing prescription drugs, including controlled substances, to keep athletes in the game is also an emerging legal issue supported by our findings. Team physicians can be held liable for negligently prescribing cortisone, pain killers, Toradol injections and other medications to facilitate sports participation during an injury.[16, 17] This is evident in the 2017 class action lawsuit filed by 13 former NFL players versus all 32 NFL teams citing the negligent use of anti-inflammatories and narcotics to return players to play. While this class action lawsuit resulted in dismissal of all claims except three before trial, players raise concerns of overprescribing narcotics leading to the opioid crisis, infringement on informed consent without disclosure of side effects and irreversible damage from continuing to play through injuries while masking pain.[16, 17]

Although the annual rate of medical malpractice lawsuits in our study nearly triples from 0.28 to 0.84 lawsuits per year, from 1971-1995 and 1996-2020, respectively, medical malpractice lawsuits against NFL team physicians are still relatively rare. However, the rise in malpractice lawsuits among NFL players has the potential to dissuade future qualified sports medicine experts from providing healthcare to these elite athletes. As professional athletes salaries continue to rise, so does the liability for future potential earnings, prompting concern from team physicians about securing adequate medical malpractice coverage in case of lawsuits resulting from career-ending injuries.[18] While diagnostic capability and medical treatment are continuing to improve, medical malpractice lawsuits from NFL players against orthopaedic surgeons are also increasing. To ensure continued high-quality care from the most qualified health care professionals, professional sports leagues and sports medicine organizations must

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advocate for protections on behalf of those healthcare professionals, who are currently under tremendous personal and professional liability.

**LIMITATIONS**

The limitations of this study are several. First, our study relied on searches of mostly non-peer reviewed publicly-available search engine and sources across the internet, which was at times incomplete. Complete data collection was limited for several reasons to include sealed records, multiple appeal processes and lack of transparency or follow-up in final rulings. All rulings, appeals and settlements are not necessarily in publicly available records or may not be stored online. While the results of our study demonstrate a steady rise in malpractice lawsuits over the last 5 decades, we cannot eliminate the possibility that this is in part related to a form of recency bias. Fewer lawsuits from the pre-internet era may have been recorded or available for review.

**CONCLUSION**

There was an increase in the number of medical malpractice lawsuits by professional athletes against team physicians in the NFL between 1971 and 2020, but the overall number remains low.**FIGURE LEGEND**

**Figure 1.** NFL medical malpractice lawsuits over time in the National Football League (NFL), 1971-2020

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**TABLES**

**Table 1.** Details of medical malpractice lawsuits in the National Football League (NFL) from 1971-2020 with data collected from publicly available sources

| Body Area | Lawsuit Complaint | Medical Condition | Lawsuit Outcome |
|---|---|---|---|
| *Knee* | • Negligently withheld medical information | • Intentional withholding of extent of knee injury | • Ruling in favor of NFL team/team physician |
| | • Negligent retention; unauthorized additional surgeries beyond consent | • Other arthroscopic procedures performed during ACLR | • Trial court granted motion to compel arbitration (case dismissed) |
| | • Mistreatment of knee injury | • Career ending injury | • Lawsuit against team physician dropped by player |
| | • Missed ACL tear | • Treated hamstring injury only; failed to diagnose chronic ACL tear | • Ruiling in favor of athlete ($3.75 million); verdict overturned after |

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|---|---|---|---|
| | | | appeal due to lack of jurisdiction |
| | • Permanent nerve damage to right leg; lack of informed consent | • As a result of arthroscopic knee surgery (unable to return to play) | • Outcome unknown |
| | • Medical negligence, cleared to return to play too soon | • Treated for PCL injury; ignored meniscus tear | • Ruiling in favor of athlete ($43.5 million) |
| | • Medical negligence for graft type; failure to address meniscus/damage to cartilage | • As a result of ACLR, resulted in re-tear (lasting permanent damage) | • Outcome unknown |
| *Spine* | • Failure to provide adequate medical care | • Spinal injury | • Lawsuit dismissed in court |
| | • Medical negligence | • Cleared to return to play too soon after neck surgery (resulted in second surgery) | • Ruiling in favor of athlete ($650, 000) |
| | • Improper treatment of neck injury | • Failed to recommend surgery | • Ruling in favor of team physician |
| | • Missed nondisplaced cervical spine fracture | • C1 fracture, C4-C7 multilevel disc bulge (caused career-ending ligament injuries) | • Outcome undisclosed |
| *Foot/Ankle* | • Failed ankle surgery | • Broken leg/ankle injury (permanent cartilage damage) | • Ruling in favor of athlete ($28.5 million) |
| | • Allowing to play through injury | • Worsening toe injury, career-ending injury (cartilage damage) | • Ruling in favor of athlete ($11.5 million); verdict overturned in retrial in favor of team physician |
| | • Misdiagnosis | • Fibula fracture misdiagnosed as bone bruise | • Trial court dismissed suit; verdict later reversed (unknown if settlement) |
| | • Procedure in non-sterile environment | • Drainage of a bruise in the locker room (resulted in hematoma and infection) | • Lawsuit settled ($2.2 million) |
| *Shoulder* | • Mistreatment of shoulder injury | • Irreversible damage, failure to disclose | • Unknown outcome |
| *Head* | • Medical negligence | • Cleared to return to play from concussion too soon (resulting in further injury and retirement) | • Ruiling in favor of player ($1.55 million) |
| *Wrist* | • Misdiagnosis | • Misdiagnosis/mistreatment of wrist injury (leading to long term disability, inability to return to throw) | • Lawsuit against team physician dropped by player |
| *Hip* | • Medical negligence | • Groin strain with rectus femoris avulsion (unable to return to play) | • Lawsuit dismissed against team physician (workers' compensation laws) |

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| | | | |
|---|---|---|---|
| *Other* | • Misuse of painkillers | • Recurrent Toradol injections to treat ankle (aggravation of injury and long-term pain) | • Ruling in favor of NFL team |
| | | • Consumption of large quantity of NSAIDs (eventually leading to heart surgery) | • Filed lawsuits against two NFL teams; ruling in favor of one NFL team and of the athlete |
| | | • Recurrent injections to treat knee injury (aggravation of injury) | • Settlement with NFL team/team physician ($600,000) |
| | | • Recurrent injections to treat pain from broken toes (subsequent bone removal) | • Settlement with NFL team/team physician ($120,000) |
| | | • Recurrent injections to treat knee injury (aggravation of injury, withholding knowledge on the extent of injury) | • Settlement with NFL team/team physician ($1.5 million) |
| | | • Overuse of Advil, prescribed by team physician (eventually leading to kidney damage, requiring transplant) | • Lawsuit settled (undisclosed amount) |
| | • Wrongful death, gross negligence | • Heat stroke (collapse during training camp, died from complications) | • Lawsuit settled (undisclosed amount) |
| | • Negligence and fraud | • Failed to take proper precautions to prevent staph infection and lied about steps taken to prevent infection (staph infection knee from workout equipment) | • Lawsuit settled with 1 of 3 physicians (undisclosed amount); ruling dismissed for other 2 physicians (workers' compensation laws) |
| | • Severe physical pain | • Punctured lung during injection of painkiller for fracture rib | • Outcome unknown |

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